

February 23, 2026

Thomas Keane, MD, MBA
Assistant Secretary for Technology Policy,
National Coordinator for Health Information Technology
U.S. Department of Health and Human Services
Mary E. Switzer Building
330 C Street SW
Washington, DC 20201

Re: Request for Information: Accelerating the Adoption of and Use of Artificial Intelligence as Part of Clinical Care (90 Fed. Reg. 60108, December 23, 2025)

Dear Assistant Secretary Keane,

As members of Michigan Medicine and the Trust, Innovation, and Ethics Research for Responsible AI (TIERRA) program, we are submitting the following comments regarding the December 2025 Request for Information on Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care. We appreciate the Department's commitment to developing AI policy grounded in evidence and stakeholder input, and we enthusiastically support the goal of advancing trustworthy AI that improves clinical care while safeguarding patient and community interests.

Michigan Medicine, the academic medical center of the University of Michigan, is a nationally and internationally recognized leader in patient care, research, and medical education. With over 3,000 faculty across 22 specialties, it delivers high-volume, high-acuity care—operating 1,043 beds across University Hospital, the Frankel Cardiovascular Center, and C.S. Mott Children's Hospital, and caring for more than 50,000 inpatients and 2.2 million outpatients each year—while driving clinical innovation that improves outcomes for diverse patient populations. Consistently ranked among the nation's top hospitals, including top-ten recognition in multiple specialties, Michigan Medicine leverages its integrated clinical, research, and educational missions to expand access to high-quality care and inform state and national health policy that benefits Michigan residents.

TIERRA (Trust, Innovation, and Ethics Research for Responsible AI) is a University of Michigan research program dedicated to promoting responsible and ethical AI in healthcare. Through empirical research on public trust, development of transparency and accountability tools, and policy engagement with health systems and communities, TIERRA generates evidence that directly shapes how AI is designed, governed, and deployed so that patients, providers, and communities experience AI as safe, fair, and trustworthy.

The following document presents comments organized around key themes in the RFI: **regulation, reimbursement, and research and development**. These recommendations are grounded in our empirical work, including:

- Community deliberations with Michigan residents across the state on AI in healthcare.
- Development and testing of patient-informed “AI health labels” for clinical AI tools.
- Nationally-representative survey data on AI and healthcare.
- Policy engagement with health institution leaders and state/local partners on trustworthy AI frameworks.
- Ongoing evaluation of AI deployment in real-world healthcare settings.

Overarching Recommendations

Drawing on Michigan Medicine and TIERRA’s work, we recommend that HHS:

1. Make trustworthiness and public trust a central organizing principle of federal AI policy in healthcare, on par with innovation and efficiency.
2. Require basic transparency, disclosure, and communication standards for health AI, including plain-language “AI labels” for tools used in clinical care.
3. Invest in community-engaged research and deliberative methods to surface multiperspectives on AI, with particular attention to community values such as safety, privacy, and maintaining the provider-patient relationship.
4. Support systematic monitoring and evaluation of AI tools in real-world clinical environments, beyond initial validation and marketing claims.
5. Align regulatory and payment policies with these goals so that AI that demonstrably advances safety, supports community values, and builds trust is incentivized, while high-risk or opaque applications face heightened scrutiny.

Regulation: Centering Transparency, Trust and Community Values

Our research indicates that regulatory frameworks will succeed only if they reflect how patients and the public understand and experience AI in their care.

Public Perspectives Highlight Transparency, Privacy, Fairness and Human Connection

In a series of Michigan-based community deliberations, we found that members of the public consistently identified several areas of concern regarding AI in healthcare: transparency, privacy and data security, fairness in how tools are used, and potential loss of human connection or a diminishing role of the provider, and uncertainty about accountability when AI is involved in their care. At the same time, participants were hopeful that AI could improve the care they receive, advance research on rare or understudied diseases, and act as a “second opinion” that supports providers rather than replaces them.

These deliberations showed that people want assurance that AI is used in ways that are transparent, safe, and aligned with community values. Regulatory policy that fails to address these concerns explicitly risks eroding trust in both AI tools and the health systems that deploy them.

I. Recommendation: Require Patient-Facing AI Transparency through Standardized “AI Labels”

Building on deliberative work in Michigan, TIERRA developed and tested a prototype “AI Health Label” intended to communicate key information about AI tools used in clinical care in a concise, patient-informed format. In our study, participants expressed strong support for labeling that includes:

- Whether and how an AI tool is used in their care
- What data sources the tool relies on, and how privacy and data security are protected
- Evidence about benefits and risk, including performance across groups in ways that reflect community concerns about fairness
- How the tool is governed (e.g. oversight processes, accountability, opportunities for recourse)

Participants viewed such labeling as critical to building trust, supporting informed decision-making and preserving patient autonomy in settings where they might otherwise be unaware that AI is being used.

We therefore recommend that HHS:

- Establish a baseline requirement for AI tools used in clinical care provide clear, plain-language disclosure to patients, analogous to a standardized “AI label”
- Coordinate with FDA, ONC, and CMS so that labeling expectations are consistent across regulatory and payment processes, and ensure that labeling practices are consistent with civil rights, privacy, and accessibility obligations.
- Encourage or require developers and health systems to use patient-informed, empirically tested label formats, such as those derived from TIERRA’s deliberative work, instead of ad-hoc or purely marketing-driven materials.

II. Recommendation: Integrate Trust, Ethics, and Stakeholder Impact into Risk Classification

Our work emphasizes that AI risk cannot be defined only in terms of technical performance; it must also consider use cases, stakeholders, and broader system-level effects. In presentations and policy-oriented work, TIERRA has highlighted the need for ethical frameworks that account for:

- The AI lifecycle (development, deployment, monitoring, and decommissioning)
- The intended context of use (clinical, administrative, population health, research)
- Which stakeholders are most affected and how (patients, providers, health systems, communities)
- Existing patterns of mistrust in health systems and community priorities that AI may exacerbate or mitigate

We encourage HHS to integrate such dimensions explicitly into AI risk stratification and oversight. For example, tools used directly in diagnosis or treatment decisions for populations who have historically been overlooked or made vulnerable, may warrant higher scrutiny, including more robust transparency, auditing and community engagement requirements.

Reimbursement and Incentives: Rewarding Trustworthy, Community-Aligned Health AI

Our work indicates that payment policy will be a powerful lever not only for adoption, but also shaping which kinds of AI are developed and deployed.

I. Recommendation: Tie Reimbursement to Evidence of Clinical Benefit, Community Values and Trust

Our focus on monitoring AI integration and delivery and evaluating whether AI improves clinical care provides a useful template for the kinds of evidence that should inform reimbursement decisions.

In particular, we recommend that HHS:

- Encourage or require rigorous evaluation of AI tools in real-world settings, including clinical outcomes, workflow effects, and patient-reported experiences of care, before large-scale reimbursement is granted.
- Incorporate measures that reflect community values into coverage and payment decisions, such as how AI affects groups defined by demographic and social factors that communities identify as important, as well as patient perceptions of transparency and respect.
- Treat transparent communication, disclosures, and robust governance processes as positive factors in payment policy.

Such approaches would support HHS's goals of fostering public trust and reducing uncertainty, while ensuring AI is deployed in ways that measurably improve outcomes for patients and communities.

II. Recommendation: Guard Against Incentives that Push High-Risk Deployment Without Adequate Oversight

Our research has underscored that AI adoption is occurring unevenly across health systems, often driven by vendor offerings and local capacity rather than systematic assessment of benefits and risks. Participants in community deliberations voiced concern that AI might be implemented primarily to cut costs or increase throughput, without sufficient attention to patient experience, privacy, or preserving the provider-patient relationship.

HHS should therefore:

- Avoid reimbursement structures that reward volume-driven or purely efficiency-driven AI use, especially where evidence of clinical benefit and acceptability to affected communities is weak.
- Support payment models that create space for provider judgment, shared decision-making, and the ability to override or decline AI recommendations without penalty.
- Require that health systems adopting AI at scale implement mechanisms to identify and respond to patient and community concerns, complaints, or harms associated with AI use.

Research and Development: Investing in Trust, Monitoring, and Community Engagement

TIERRA stands at the intersection of research, ethics, and policy. The program aims to assess public trust in health institutions and AI, investigate how perceptions of AI affect trust, and monitor AI integration in healthcare delivery and evaluate outcomes, and influence policies to advance system-level trust.

Recommendation: Fund Longitudinal Monitoring of AI Integration in Health Systems

Our work suggests that HHS should treat health AI not as a one-time deployment decision, but as an ongoing sociotechnical transformation that requires continuous monitoring and adaptation. In addition, our work to evaluate tools used in clinical settings, track trends and changes in public or patient perspectives, and define the landscape of AI use in health systems point toward several priorities for federal R&D support.

We recommend that HHS:

- Support longitudinal, multi-site studies of AI implementation in diverse health systems, with attention to heterogeneity in populations, infrastructure and governance.
- Invest in methods and tools for continuous evaluation of AI, including post-deployment auditing, performance drift detection, and mechanisms to incorporate patient and clinician feedback over time.
- Encourage cooperative agreements and public-private partnerships that bring together health systems, community organizations and academic centers like Michigan Medicine and TIERRA to align technical innovation with ethical and social priorities.

A Roadmap for Trustworthy AI and Health Systems

We encourage HHS to view trustworthiness not as a constraint on innovation, but as the foundation of which sustainable and beneficial AI adoption must stand on. By centering trustworthy principles, HHS can accelerate AI adoption in ways that genuinely improve health outcomes and strengthen, rather than erode, trust in AI systems and health systems.

We appreciate the opportunity to comment on this RFI and would welcome future collaboration as HHS refines its strategy for AI in clinical care.

If you have questions or would like to further discuss our comments, please contact Dr. Jodyn M. Platt at (734) 936-1644 or jeplatt@med.umich.edu. Thank you for your consideration.

Sincerely,

Jodyn M. Platt, PhD, MPP

Associate Professor, Learning Health Sciences & Health Management and Policy
Director, Trust, Innovation, and Ethics Research for Responsible AI (TIERRA)
University of Michigan | Medicine

Michael L. Burns, PhD, MD

Clinical Associate Professor of Anesthesiology
Associate Chief Medical Informatics Officer of Artificial Intelligence
Chair of Clinical Intelligence Committee
University of Michigan | Medicine